

BREAST BIOPSIES ARE NOT DONE LIGHTLY: A RETROSPECTIVE REVIEW OF 1307 BREAST BIOPSIES

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BACKGROUND

Core needle biopsy (CNB) is the standard method of obtaining histopathological diagnosis. The ACR BI-RADS® Atlas, 5th edition recommends a positive predictive value 2 (PPV2) as a quality indicator, the benchmark being $\geq 25\%$.

This audit evaluated the PPV2 of CNB, to determine whether CNB were performed in appropriately targeted patients according to clinical judgement and radiological findings in Picaso Hospital.

OBJECTIVES

- ★ Determine overall PPV2 of CNB in Picaso Hospital
- Determine PPV2 of each BI-RADS category & BI-RADS 4 subcategory
- Determine monthly trend of overall PPV2

METHODS

Retrospective single-centre audit
October 2024 – June 2026

Inclusion criteria:

- Ultrasound-guided CNB
- Clinically-guided CNB

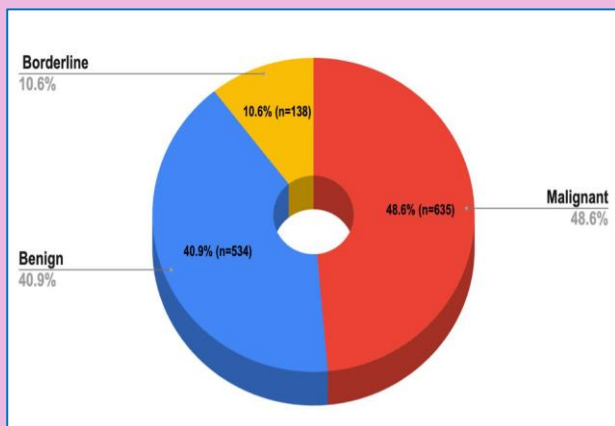
Exclusion criteria:

- FNAC
- Skin biopsy
- Non-histopathological investigations

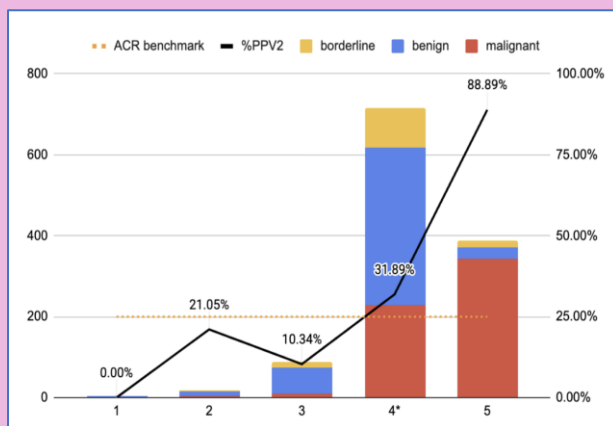
Data collected: age, BI-RADS, histopathological diagnosis, UK NHSBSP classification

PPV2 = malignant biopsies / total biopsies x 100%

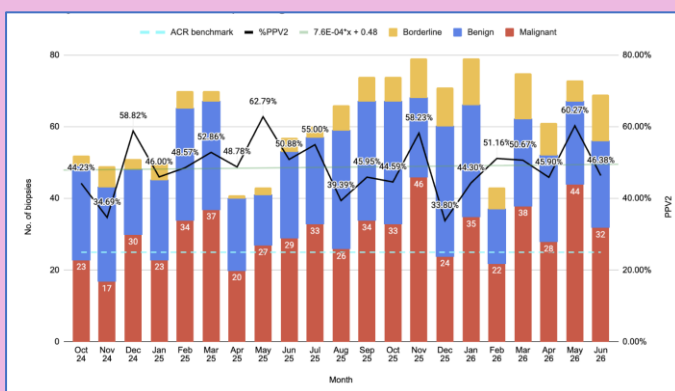
RESULTS



Histopathological outcomes



PPV2 of each BI-RADS category



Monthly trend overall PPV2

48.6%
OVERALL PPV2

1307
BREAST BIOPSIES

51.5 ± 12.6 yrs
MEAN AGE

CONCLUSION / TAKE-HOME MESSAGE

Overall PPV2 is 48.6%

Exceeding ACR benchmarks of $>25\%$

Patients are appropriately selected for biopsies

Good integration of clinical judgement with radiological findings

Determine malignant potential of borderline lesions

By investigating the rate of malignancy conversion of borderline lesions

REFERENCES

1. UK NHSBSP B-classification / EWG-BPS European Standards
2. ACR BI-RADS 5th Edition

ACKNOWLEDGEMENTS

No conflicts of interest to report